DR Care Solutions

Danielle Robertson Consulting Pty Ltd trading as DR Care Solutions

ABN 44 605 748 392 ACN 605 748 392
3408/1 Sergeants Lane, St Leonards NSW 2065

Overview for Residential Aged Care Placement

CLIENTS NAME: **VICTORIA 'JUNE' BALDWIN known as JUNE**
DOB: **29.11.1926 (98 years of age)**
CLIENT ID: **DRCS 0543**

This overview is a summary completed for all clients requiring Placement into Residential Respite or Residential permanent aged care, assisted living or group home. The overview is undertaken by a consultant from DR Care Solutions. It's used as a guideline outlining the background and needs of a specific client(s).

Issues covered:	**Background and relevant history;** Susie Lapin, one of June's daughters reached out to Danielle from DR Care Solutions on Wednesday 12th March 2025 to discuss her mother's ageing care and living needs. We discussed her progressive health issues, her advancing years and the need to relocate into a supported living environment. We arranged a Zoom catch up with Susie, her brother Richard and June on Wednesday 19th March 2025, to gather information for this summary. Susie was referred to Danielle by her friend, Dof Dickenson (a previous client of DR Care Solutions). From June's GP Health Summary, we understand that in 2003, June had an isolated episode of Atrial Fibrillation and that re-occurred in 2014. In 2006, she had shoulder surgery, a hip replacement in 2007 and a vertebrae fracture in 2007. Also, around 2007, she was diagnosed with Macular Degeneration, the dry type initially, however over time it's developed into the wet type. Additionally, she's had B12 and Vitamin D deficiencies, osteopenia (reduced bone density) and osteoarthritis in her knee and hip. In 2021, she developed Thyrotoxicosis (a treatable condition that happens when you have too much thyroid hormone). June was driving up until about 8 years ago. June underwent a total left hip replacement on 7th December 2024. She's recovered extremely well with no pain whatsoever and healed quickly. **Physical health including mobility, continence:** For June's advanced years, she has no chronic illnesses. She's been living alone in her own home in Stanmore with external support services coming in.
• Physical health including mobility, continence, • Cognitive health • Sensory (sight, hearing, oral, feet) requirements • Activities of daily living • Medications, allergies & vaccinations • External assistance - including Allied Health • Emotional & mental health • Social background, networks and Relationships • Likes & dislikes	

● Cultural linguistic and Spiritual needs ● Estate Plan/ Advice and Life Ready Register ● Funeral arrangements ● Financial Plan/ Advice ● My Aged Care ● Criteria for next home	June recently (about a month ago) injured and 'slashed' her leg on a watering can in her garden. She ended up in hospital requiring four stitches and having it appropriately dressed. The wound is healing nicely, and dressings are changed twice a week by a community nurse. June uses two 'hiking' sticks for mobilising outdoors. She knows her home inside out. She furniture and wall surfs throughout the house. She doesn't use a wheelie walker nor a standard walking stick inside. June remains frail. Her skin is fragile and she's prone to bumps, bruises and scratches. She takes her time ambulating. June remains fully continent and hasn't any need to wear pads. She has urinary frequency however no urgency nor leakage. Her bowels are managed independently. **Cognitive health:** June advised she is forgetful at times. She was articulate and able to discuss her current health issues and what she's looking for in her next home. There is no evidence of cognitive decline or dementia, possible age-related memory loss. **Sensory:** June's primary health concern now, is her impaired vision, which is rapidly worsening due to macular degeneration (wet type) where she needs regular injections (every 4 – 6 weeks) to slow down the decline. June doesn't wear glasses; she doesn't need nor wear hearing aids. She has no dentures however she does have a tooth implant. She visits a professional podiatrist every 6 weeks. She has hammer toes on her two middle toes. She doesn't wear orthotics nor special shoes. They manage her nail and foot care. **Activities of daily living:** June is assisted by carers with her showering three days a week. She can dress herself independently and manage her grooming and toileting without assistance. She visits the hairdresser every six weeks for a haircut. Her hair is washed when she showers. June wakes very early (ie: between 5am and 6am). Her carers arrive at around 7am – 7.30am. She makes all her own meals independently. She's not a great sleeper and advised it was probably due to having children and being alert and light sleeping when raising them as well as being on a farm when she was younger. **Emotional & mental health:** June is a little nervous about the move from her home into a new environment which is understandable after living in her own home, alone and independently for so long. She isn't anxious nor depressed. She appeared philosophical about her situation.

Medications, vaccinations & allergies: June has no known allergies. All June's medications are in their original boxes however we suggest prior to her relocation into a supported living environment that she considers requesting her pharmacist to provide all medications into a Webster-Pak® with at least 7 – 14 days' worth of medications.

Based on June's current GP Health Plan her medications and supplements include:

- Apixaban (Blood thinner to manage Atrial Fibrillation)
- Bilberry (for Macular degeneration management)
- Celestone cream (for management of dermatitis)
- Digoxin (to manage Atrial Fibrillation)
- Elocon (for management of itchy skin on ears)
- Fish oil (Supplement for Macular degeneration management)
- Macu-vision (for Macular degeneration management)
- NEO - B12 (Supplement for B12 deficiency)
- Ostelin Vitamin D (Supplement for Vitamin D deficiency)
- Panadeine Forte (For pain management of Osteoarthritis)
- Prolia Syringe (to manage Osteopenia ie: low bone density)
- Rozex Gel (to manage inflammation in relation to rosacea)

Social background, networks, and relationships: June was born in Sydney and at the age of 7 years her family moved to Oberon, in the central tablelands of NSW. It is a very pretty area. June grew up as a 'farm girl' and remembered riding her horse to school, even when it was snowing. She attended the local Catholic School (St Joseph's Catholic School) for her primary years. She didn't attend high school. As a young girl she loved to horse ride and played basketball.

June was very strong physically growing up in the bush. They had an orchard, killed their own meat, raised chickens and grew their own vegetables. They went to town once a week for supplies. Until she came to Sydney she was eating fully organic foods. Fresh air and exercise have probably contributed to June's long life. June was a wonderful sewer and knitter of rugs. She could turn her hand to anything.

June assisted her parents around the farm. She obtained work in the local pharmacy in Oberon as an Assistant Dispenser of medications. At the age of 19 years, she relocated to Sydney and worked at the Double Bay Pharmacy. She met an Englishman, fell in love and they married. They lived initially in Sydney and then Orange for 30 years.

June had five children: Richard, Susan, Jennifer (known as Thankam), Jane and Penelope. Richard lives in Hong Kong; her daughters live in Sydney on the Northern Beaches and in the Eastern Suburbs. June has 6 grandchildren and 3 great-grandchildren.

	June was an accomplished homemaker, she ran a beautician show on television, she was a very good cook. When June was 50, her husband passed away and she sold the farm in Orange and relocated to Sydney. She decided to attend university to study Nursing (and came second in the state). She worked at the old Prince Henry Hospital which was very physical work. She decided to change employers and went to the RPA (Royal Prince Alfred Hospital) and worked as a theatre nurse. She nursed for 20 years retiring at the age of 70. After June retired, she has and continued to participate in a Poetry group who visit her at home and read poetry and discuss philosophical books. Over the years the group has changed although the group has been together for fifteen plus years and consists of around eight people (men and women). She gets a lot of pleasure from this group. **Likes & dislikes:** June is a good eater and drinks plenty of water which is great as many older Australians suffer dehydration. She enjoys eating chicken, fish, chops and steamed vegetables. She loves olives, cheese and all sweets (ice-cream, vanilla yoghurt, fruits, chocolates and desserts). She enjoys her tea and coffee (Moccona Instant coffee) and a flat white or cappuccino when she goes to a café. June smoked for a year when she was 20 as it was the 'thing to do'. She used to occasionally have an alcoholic drink however she doesn't drink alcohol now. June used to play and enjoy tennis as an adult. June loves being in the bush, walking, gardening, socialising with friends, going to the opera and theatre and was an avid book reader. She used to read voraciously. Given her vision impairment, she listens to MP3 and CD's of talking books. She loves good jazz and music from her era. June doesn't like noisy or polluted places. **Cultural, linguistic & Spiritual needs:** Raised as Catholic, June advised she isn't religious nor practising. She can't speak any other languages and didn't learn a musical instrument. **Estate Plan/ Advice:** We understand that Enduring Power of Attorney is in place for financial decisions should June lose capacity. Richard and Susie are joint Powers of Attorney for June. Susie and Thankam (Jennifer) are listed as joint Enduring Guardians for health, care and accommodation decisions for June should she lose capacity. Richard lives in Hong Kong and therefore June felt it best for two of her daughters living in Australia be her enduring guardians. We haven't sited copies of these documents; however, June will need to supply copies when she relocates into a supported living environment.

	Advanced Care Directive: Below we include a special DR Care Solutions link to Touchstone Life Care for building an Advanced Care Directive. You are welcome to use this or ask your GP for a paper-based document if it's preferred. This is usually stored in the client's GP Health Summary and hospital records should something occur, the medical profession adheres to the requests if the person is unable to communicate their wants, needs and wishes. Touchstone Life Care - A DR Care Solutions Partner **Funeral arrangements:** This was not discussed with June; although we recommend that it's a topic that should be discussed. The home June selects will be requesting this information. We suggest that if you are unsure of who to select as a Funeral provider, we work with several providers in the Family Business sector who have obtained wonderful feedback on the service provision. We can make a warm introduction (if requested). **Life Ready Register: For** DR Care Solutions clients, we provide a copy of the Life Ready Register, a depository to keep all your affairs in one place. We suggest that you download this and start completing it. No matter what age, everyone can commence building this out and sharing it with their trusted network, their Powers of Attorney and Enduring Guardians as well as their Executor of their Will. Life Ready Register™ **Financial Plan / Advice**: June is on a full Government Pension although she owns her own home and will be a RAD (Refundable Accommodation Payment) Payer. She will likely sell her home to fund the RAD for residential aged care and invest the balance to pay for her ongoing care and other expenses. She may lose her government pension. We suggested that the family obtain advice from an Aged Care Accredited Financial Adviser. Danielle introduced Susie to a Financial Adviser who is aged care accredited on 12th March 2025. They've already spoken and discussed the best options of funding June's aged care. **My Aged Care:** June has had an ACAT assessment on 1st June 2022 where she was provided with a level 3 home care package and the referral codes for high level residential respite care and residential permanent care. She has been using her level 3 Home care package for personal care (showering), domestic assistance, social support, podiatry, ad hoc gardening assistance and nursing any open wounds she may have. It was extremely wise to have arranged the referral codes for residential care in 2022. Referral codes for residential aged care are as follows:

	Residential Respite High Care: 2 – 144 624 122 597 **Residential permanent care: 2 – 144 624 122 685** **Criteria for next home:** The Inner West or Eastern Suburbs of Sydney are June's desired location. Family members also live on the Northern Beaches however, June has requested where she feels comfortable living. A strong focus on care is requested with a company that is 'well run'. Other residents with good cognition will be something we will be asking about on the tours. Residents with frailty or visual impairment would be ideal so that June has others to converse and socialise with. The selected home needs to be light, bright and airy with the ability for June to obtain fresh air either with an opening window or a balcony or courtyard where she can bring a few pot-plants to tend and enjoy the fresh air. A home that has lovely gardens would be ideal as June has a beautiful garden which she tends at home which is full of flowers. Her passion is gardening and raised beds of plants would be ideal for her and ultimately will enhance her quality of life.
Actions required (for the client to achieve these goals)	Danielle from DR Care Solutions is to locate a minimum of FOUR Aged Care homes **two in the Eastern Suburbs and two in the Inner West of Sydney** who can manage June's care needs now and into the future. They will need to be able to manage all June's care requirements (outlined herein), have the availability for respite with a view to permanent care and be able to provide top quality care, support services with appropriate social interaction and activities for her to participate in. Her care needs currently, are quite low however likely to increase as she ages and her condition progresses. Once these have been located, pre-vetting, due diligence, clinical acceptance, and tours will begin. We have diarised **Monday 31st March 2025** (in the afternoon) for the **Inner West tours** and **Friday 4th April 2025** (in the afternoon) for the **Eastern Suburbs tours** as a possible date for tours of local homes, with a view to relocate as soon as a room in the selected home is available for admission. June is keen to relocate as quickly as possible given the deterioration of her eyesight and her advancing requirements for support.

Who will be involved: (Guardian, solicitor, financial planner / advisers, family members)	• June Baldwin, Susie Lapin, Richard Baldwin and any other family members • Danielle Robertson from DR Care Solutions • GP, Geriatrician, Ophthalmologist and any other medical specialists involved in June's care. • Financial Adviser (Aged Care Accredited) • Estate Planner / Lawyer (review of Estate Planning documents on a regular basis).
Review of Summary for Placements and the date the Family need to determine next steps:	Tuesday, 25th March 2025

Signature (Consultant):  Date: Saturday 22nd March 2025

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